

Everyone Around an Autistic Working Adult

The cumulative guide for the whole team around an autistic adult in employment — family & partners, trainers & mentors, coaches/gym leaders, doctors & occupational-health nurses, job coaches & support workers, colleagues & employers, and friends. *An extended reference sheet; pair it with the single-counterpart quick guides.*

The one idea

Autism, through **monotropism**, means an adult's attention locks into a few deep, intense "tunnels." In the right job that focus is a genuine superpower — depth, reliability, pattern-detection, honesty. In the wrong environment (open-plan noise, constant interruption, office politics, vague expectations) it drives masking and **autistic burnout**, which is distinct from ordinary occupational stress and depression. Many working autistic adults — especially women — were identified only in adulthood. Whatever your role, the levers are the same: protect focus, be clear and predictable, communicate in writing and literally, and take burnout as the serious, distinct thing it is.

What helps across every setting

- **Protect focus and the special interest** — structure work around fewer, deeper "tunnels, not tasks"; minimise interruptions and meetings; allow flow time.
- **Be clear and explicit** — written briefs, defined deadlines, named expectations; say the "unwritten rules."
- **Communicate in writing and literally**; allow asynchronous options; give processing time; don't read a pause as resistance.
- **Lower sensory and social load** — quieter spaces, optional social, fewer forced group demands.
- **Take burnout seriously** — chronic exhaustion, loss of skills and reduced tolerance need *reduced load and unmasking*, not "push through"; reasonable adjustments, time off or a role change are recovery, not weakness.
- **Centre their expertise** — the person is the expert on what helps; reasonable adjustments (Equality Act / ADA) are a right.

What to avoid across every setting

- Vague briefs, last-minute changes, surprise "quick chats," or "you should just know" expectations.
- Equating eye contact, small talk or "visibility" with commitment or competence.
- Treating burnout as depression or laziness; pushing the person to mask more.
- Equating a masked, competent hour with being "fine," or "not complaining" with "not suffering."

Quick notes for each person around the worker

Family, partners & friends

Believe and validate a late identification; back accommodations and rights. Take burnout seriously — reduced demands and unmasking beat “push through.” Help with practicals if asked; respect autonomy, including a role change or reduced hours.

Trainers & mentors

Put materials and instructions in writing, in advance; offer alternatives to forced group work; one instruction at a time with reasons. Play to depth and interest; check in literally on load.

Coaches, gym & club leaders

Offer written plans and predictable structure; one cue at a time; respect headphones, quiet and routine as regulation. Be alert to burnout; a flexible, low-demand membership helps through hard patches.

Doctors & occupational-health nurses

Offer the AASPIRE *AHAT* and written-first options; recognise **autistic burnout** (distinct from depression); broker reasonable adjustments. Start medications low and titrate slowly. Assess for suicidality and co-occurring ADHD, anxiety, sleep and GI conditions.

Job coaches & support workers

Help articulate reasonable adjustments; structure the role around “tunnels, not tasks”; be the predictable, safe check-in. Take burnout seriously and centre their expertise.

Colleagues & employers

Keep social optional and low-key; protect focus; give literal, kind feedback. Make reasonable adjustments routine. Watch for burnout and signpost OH/employee assistance.

Friends & peers

Keep friendship low-key and optional; say what you mean; connect through shared interest. If they seem to be burning out or talk about hopelessness, signpost help and don't sit on it alone.

When to get more support

Watch for **autistic burnout** (chronic exhaustion, loss of skills, reduced tolerance) — it's linked to **suicidality** and is distinct from ordinary work stress and depression. Address co-occurring **ADHD (AuDHD), anxiety, depression, sleep and GI conditions** with autism-informed clinicians. Reasonable adjustments, reduced hours or time off are recovery, not weakness.

If the person is a woman

Women are over-represented among the “lost generation” of late-diagnosed adults and mask most heavily. Take a lifelong pattern of social/sensory difference and private exhaustion seriously across every setting, even with a “high-performing” exterior.

About this guide

AI-assisted, derived from this project's research drafts — the **Lifespan Practitioner & Health-Care guide** (Part 3.4), the **Family guide** (Part 4) and the **Monotropism** brief. Uses identity-first language. **Informational — not medical advice or a diagnostic tool.** Fit it to the individual.

Key sources. Murray, Lesser & Lawson (2005); Murray, F. (2023); Dwyer (2025); Knight (2021, *tunnels not tasks*); Booth (2021, work & autistic people); Raymaker et al. (2020, burnout); Mantzalas et al. (2022); Pearson & Rose (2021, masking); Kelly Mahler (2024); AASPIRE AHAT; Michaud & Harvey (2025). Full citations are in the source drafts.